



P11 · 60 PAGES, UNDATED

A Newborn Sleep Log

Sixty pages of sleep-and-feed tracking for the first year.

One-handed in the dark, no shame, no math. Pattern visualizer for the doctor's appointment. Plus a "what we tried" log so you stop trying the same thing three times.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A note before you start

Around week three of my second baby, someone in my family asked me how he was sleeping. I had no answer. Not a “he sleeps four hours then wakes” kind of no-answer. A “I genuinely have no idea what happened last night” kind of no-answer. I had been awake too many times in a row to keep a coherent count.

That was the night I started writing things down on the back of an envelope on the nightstand. Just times. Just to have something. By the end of the week I had a real picture for the first time, and could finally answer the question.

This log is the version of that envelope, built for the dark.

It is not a sleep training program. It does not tell you what to do. It does not score your baby’s sleep against any chart. It only asks you to write down what actually happened, one line at a time, so you stop trying to guess.

Soothemade

This log is a tracking tool, not medical advice. If your baby is not gaining weight, is sleeping unusually much, is hard to wake for feeds, or you are worried, call your pediatrician. The trusted numbers are at the back.

How to use this log

- Use one daily page at a time. Sixty pages total, undated. Use them as a stretch of two months, or spread them across the year, or skip weeks.
- Fill in as you go, not later. Times are easier to capture in the moment than to reconstruct.
- One-handed handwriting is fine. Sideways notes are fine. Cross-outs are fine.
- The page is not a scorecard. The baby is not a project. You are looking at patterns, not performance.

A note on safety

The American Academy of Pediatrics recommends:

- Babies sleep on their back, on a firm flat surface, until age one.
- No loose bedding, pillows, bumpers, or soft toys in the sleep space.
- Room-sharing (not bed-sharing) for at least the first six months.
- A pacifier at naps and bedtime (after breastfeeding is established) may reduce SIDS risk.

If you have decided to bed-share for any reason, look up the Safe Sleep Seven guidelines and read them before you do. We are not the place to ask whether it is safe in your specific situation. Your pediatrician is.

The daily log

You will see this page sixty times in the printed log.

NOTABLE TODAY (illness, travel, growth-spurt sense, vaccine, etc.):

[illegible]

Sleep total: _____ hours

Longest single stretch: _____ hours, between _____ and _____

Daytime sleep (rough): _____ hours

Nighttime sleep (rough): _____ hours

Total feeds: _____

Wet diapers: _____

Dirty diapers: _____

ONE LINE ABOUT TODAY

The grid is the work. The totals at the bottom can wait until morning, or never. Skip them if you'd rather. The totals are for whoever you are showing the log to, not for you.

The weekly snapshot

Once a week, fill in the snapshot. There is space for one snapshot per week in the printed log.

A pediatrician or sleep consultant can do useful things with these. You don't have to interpret them. You just have to write them down.

WEEK OF _____ to _____

LONGEST STRETCH THIS WEEK ____ hours on _____

AVERAGE SLEEP PER 24H ____ hours

NIGHT FEEDS PER NIGHT, AVG ____

NAPS PER DAY, AVG ____ (each averaging ____ min)

HARDEST NIGHT THIS WEEK

Date: _____ What happened: _____

EASIEST NIGHT THIS WEEK

Date: _____ What was different: _____

PATTERN OR CHANGE I NOTICED

WHAT I WANT TO TRY NEXT WEEK

The what-we-tried log

A separate section of the printed log, with twenty rows. Filled in over time. The point is so that you stop trying the same thing three different weeks and wondering whether it worked.

WHAT WE TRIED	WHEN	DID IT HELP?	NOTES
White noise level up	_____	☐ y ☐ n ☐ ?	_____
Swaddle on / off	_____	☐ y ☐ n ☐ ?	_____
Room cooler (___ F)	_____	☐ y ☐ n ☐ ?	_____
Room warmer (___ F)	_____	☐ y ☐ n ☐ ?	_____
Black-out curtain	_____	☐ y ☐ n ☐ ?	_____
Last-feed earlier	_____	☐ y ☐ n ☐ ?	_____
Last-feed later	_____	☐ y ☐ n ☐ ?	_____
Dream feed at 10pm	_____	☐ y ☐ n ☐ ?	_____
Pacifier offered	_____	☐ y ☐ n ☐ ?	_____
Sleep sack switched	_____	☐ y ☐ n ☐ ?	_____
Bath before bed	_____	☐ y ☐ n ☐ ?	_____
Shorter wake window	_____	☐ y ☐ n ☐ ?	_____
Longer wake window	_____	☐ y ☐ n ☐ ?	_____
Earlier bedtime	_____	☐ y ☐ n ☐ ?	_____
Later bedtime	_____	☐ y ☐ n ☐ ?	_____
Co-sleeping (Safe-7)	_____	☐ y ☐ n ☐ ?	_____
Cry-it-out (5/10/15)	_____	☐ y ☐ n ☐ ?	_____
Pick up / put down	_____	☐ y ☐ n ☐ ?	_____
Solid foods at ___ mo	_____	☐ y ☐ n ☐ ?	_____
Other: _____	_____	☐ y ☐ n ☐ ?	_____

If you only ever fill in three rows of this, that is still enough to stop you trying the same thing in two months and forgetting that you already tried it.

The regression pages

Sleep regressions show up almost universally at certain ages. Knowing they are coming does not make them easier, but it does mean you do not have to think “what did I break?” each time. You did not break anything. The baby’s brain is doing a developmental sprint.

There is a one-page reference for each regression in the printed log.

The four-month regression

What happens: around 3-5 months, the baby’s sleep cycles mature. Light sleep becomes lighter. They start waking between cycles instead of bridging them.

What’s normal:

- Many more night wakings, often hourly
- Naps that suddenly seem to last only 30 minutes
- A baby who fell asleep independently a month ago now needing help again
- Two to six weeks of this

What helps:

- A consistent wind-down (bath, lights low, the same song)
- Drowsy-but-awake placement, if you have the energy
- A dim, quiet, cool room
- Patience (not the inspirational kind, the resigned kind)

What does not help:

- Forcing a sleep training method during the regression itself
- Researching “is something wrong with my baby” at 3am

- Comparing notes with the friend whose baby slept through this

The six-month regression

Often coincides with starting solids, sitting up, and teething. Many of the same symptoms. Many of the same solutions.

The eight-to-ten-month regression

Coincides with separation anxiety arriving for real. The baby is now object-permanence aware and knows you exist even when you leave the room. They protest. Loudly.

What helps:

- A short, predictable goodbye ritual
- Brief absences during the day, not just at bedtime
- A lovey or comfort object (after 12 months, per AAP)
- A repeat of the consistent wind-down

The twelve-month regression

Often coincides with first steps and language explosion. Often coincides with dropping to one nap (which usually doesn't fully settle until around 18 months).

What helps:

- Resisting the urge to drop the nap too early. Most 12-month-olds still need two naps.
- A consistent bedtime, even if the morning nap is moving around.

The nap reset

A two-page reference for when something has shifted and the daytime is in disarray. Print it. Tape it to the fridge if you have to.

When to reset

- You don't recognize what the days look like anymore
- The baby is overtired by 4pm every day
- Naps are happening but bedtime has become a nightmare
- A vacation, illness, or travel has scrambled the schedule

How to reset (5 days, gently)

Day 1, observe. Just write down what is happening. Do not try to change anything. You're gathering data.

Day 2, fix only the wake-up time. Same time, give or take 15 minutes. Light the room. Out of the crib.

Day 3, hold the wake-up time AND offer the first nap at the same time each day. Even if the baby fights it. Even if it's a 20-minute nap.

Day 4, hold those two anchors, and now anchor bedtime. Pick a time and start the wind-down the same way each night.

Day 5, observe again. The pattern is usually starting to reform.

You are not training. You are scaffolding. The baby finds the rhythm in the structure you offer.

Is this normal? A monthly reference

For each age, what a pediatrician would describe as a typical range. These are wide ranges, not targets. Your baby is in the range or near it. You don't need to optimize.

0-1 month

- Total sleep: 14-17 hours per 24-hour day
- Longest stretch: 2-4 hours, almost always interrupted by feeding
- Day vs night: undifferentiated
- Naps: many, irregular, short

1-3 months

- Total sleep: 13-16 hours
- Longest stretch: 4-6 hours starts appearing, especially toward 3 months
- Day vs night: starts to emerge around 6-8 weeks
- Naps: 4-5 per day, shorter to longer

3-6 months

- Total sleep: 12-15 hours
- Longest stretch: 6-8 hours by 5-6 months for many (NOT all)
- Day vs night: established
- Naps: 3-4 per day, more consistent length

6-9 months

- Total sleep: 12-14 hours

- Longest stretch: 8-10 hours possible
- Naps: 2-3 per day, settling into morning and afternoon

9-12 months

- Total sleep: 11-14 hours
- Naps: 2 per day, often around 9-10am and 1-2pm
- Night feeds: often phased out (medically, not necessarily by your baby)

Where to ask

Your pediatrician. Not the internet. Not a sleep consultant on social media. Bring this log to a real provider if you have questions.

A partner handoff page

For the nights or naps when the other parent takes over. Print it. Hand it over. Stop re-explaining at midnight.

WHAT THE BABY USUALLY DOES AT THIS AGE (___ months)

Last feed before bedtime: usually around _____

Bedtime: roughly _____

First night waking: typically around _____

Last waking: typically around _____

THE WIND-DOWN ROUTINE (in order)

1. _____

2. _____

3. _____

4. _____

THE CURRENT MUST-HAVES

☐ Swaddle / sleep sack (which one: _____)

☐ White noise (level: _____)

☐ Pacifier

☐ Room temperature: _____ F

☐ Lights off / dim

☐ _____

THINGS WE'VE LEARNED RECENTLY THAT YOU SHOULD KNOW

IF THE BABY WAKES IN THE NIGHT

Step 1: _____

Step 2: _____

Step 3 (if still awake): _____

WHEN TO WAKE ME UP (the bar is high)

☐ A fever (above 100.4°F under 3 months = always)

☐ Vomiting

☐ Won't stop crying after 30+ minutes

☐ I just feel like something's wrong

Numbers to keep

PEDIATRICIAN

Name: _____ Number: _____

After-hours line: _____

SLEEP CONSULTANT (if any)

Name: _____ Number: _____

LACTATION CONSULTANT

Name: _____ Number: _____

US NATIONAL BREASTFEEDING HELPLINE

1-800-994-9662 (Mon-Fri)

US MATERNAL MENTAL HEALTH HOTLINE

1-833-852-6262 (24/7, free, confidential)

US SUICIDE & CRISIS LIFELINE

988 (call or text, 24/7)

POISON CONTROL (US)

1-800-222-1222 (24/7)

NIGHTS WHEN I AM TOO TIRED TO PARENT WELL

If you find yourself unable to keep your eyes open and you are the only adult, put the baby in a safe sleep space (back, firm flat surface, nothing else in the crib) and go lie down for fifteen minutes. A fed, dry baby in a safe space is fine for fifteen minutes. You are not.



From *Soothemade Notes*, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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